SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pertuzumab + trastuzumab maintenance

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive metastatic breast cancer · maintenance · NCCP 00726 · version 3a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This maintenance treatment continues HER2 blockade to help control metastatic or unresectable recurrent HER2-positive breast cancer after chemotherapy.

Schedule
Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pertuzumab
HER2-TARGETED ANTIBODY

- ☐ Diarrhoea and dehydration
- ☐ Infusion / allergic reaction
- ☐ Heart pumping-function reduction
- ☐ Rash and fatigue

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions, the planned monitoring and who to contact. Review the full longitudinal pathway and confirm which conditional treatments apply.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00726 v3a.

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